Gemcitabine 1250 and cisplatin: nasopharyngeal cancer

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1: combination

Active medicines: Gemcitabine + Cisplatin

Emetogenic risk: high

Medicine	Dose / route	When
Aprepitant	125 mg PO OD	Day 1 Clinician/pharmacy draft; reconcile to the individual prescription.
Ondansetron	16 mg PO OD	Day 1 Clinician/pharmacy draft; reconcile to the individual prescription.
Dexamethasone	12 mg PO OD	Day 1 Clinician/pharmacy draft; reconcile to the individual prescription.
Olanzapine	5 mg PO OD	Day 1 Clinician/pharmacy draft; reconcile to the individual prescription.
Aprepitant	80 mg PO OD	Days 2-3 Clinician/pharmacy draft; reconcile to the individual prescription.
Dexamethasone	8 mg PO OD	Days 2-4 Clinician/pharmacy draft; reconcile to the individual prescription.
Metoclopramide	10 mg PO TDS	Subsequent days / as directed Clinician/pharmacy draft; reconcile to the individual prescription.
Olanzapine	5 mg PO OD	Days 2-4 Clinician/pharmacy draft; reconcile to the individual prescription.

Additional phase-specific support

- Gemcitabine runs over 30 minutes, followed on Day 1 by cisplatin over 2 hours. Cisplatin requires prescribed fluids before and after treatment. Day 8 is gemcitabine alone.
- Day 1: full blood count, kidney and liver tests. Day 8: full blood count and creatinine. Hearing assessment and measured kidney clearance are arranged when indicated.
- Confirm the full treatment sequence and any separate radiotherapy plan. The linked NCCP document gives 4–6 cycles; this is not the three-cycle 00903 induction schedule.
- The local CUH source is retained as a reference list, not as the national antiemetic standard. Nystatin/chlorhexidine are displayed as local mouth-care adjuncts, not universal antiemetic or mucositis prophylaxis. Verify current local oral-care policy.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00517 v2 · pTreatment / supportive care · NCCP regimen and NCIS antiemetic guidance

Sources checked 2026-11-11 · SACTCheck v0.84.49 · Regimen-level drafts; prescriber and oncology-pharmacy verification required